Document Control

Title **Safe preparation and transfer of infusions between Theatres and Critical Care SOP**				
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Directorate Planned Care			**Department** Critical Care	**Team/Specialty** Critical Care
Version	**Date Issued**	**Status**	**Comment / Changes / Approval**	
0.1	April 2019	Draft	Initial version for consultation	
0.2	July 2019	Draft	Amendments made following consultation process with anaesthetic department	
1.0	August 2019	Final	Approved at Planned care divisional governance meeting	
2.0	Dec 2020	Final	Amendment made to appendix following approval at critical care governance meeting.	
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Document Class Standard Operating Procedure			**Target Audience** Critical Care and Theatres	
Distribution List Critical Care, Emergency Department and Theatres			**Distribution Method** Trust's internal website	
Superseded Documents				
Issue Date August 2019	**Review Date** August 2022		**Review Cycle** Three years	
Consulted with the following stakeholders: • Anaesthetic department • Critical care • Theatres • Emergency department • Pharmacy department			**Contact responsible for implementation and monitoring compliance:** Lucy Bates Clinical Matron Critical care **Education/ training will be provided by:** Critical Care/anaesthetic team	
Approval and Review Process • Critical Care Development Committee • Anaesthetic Governance • Planned Care Divisional Governance				

Local Archive Reference G:\\ Safe preparation and transfer of infusions between Theatres and Critical Care SOP **Local Path** Critical Care\ Safe preparation and transfer of infusions between Theatres and Critical Care SOP **Filename** Safe preparation and transfer of infusions between Theatres and Critical Care SOP	
Policy categories for Trust's internal website (Bob) Critical Care	**Tags for Trust's internal website (Bob)** Critical Care, Theatres

CONTENTS

Document Control 1
1. Background 4
2. Purpose 4
3. Scope 4
4. Location 4
5. Procedure 4
6. Associated Documentation 5

1. Background

Ensuring standardisation of medication preparation and safety of medication on transfer from Theatres to Critical Care is paramount in maintaining patient safety, reducing incidents of harm and providing consistency and continuity of care. In addition this SOP provides traceability and an audit trail for medication requests and transfer between clinical departments.

2. Purpose

2.1. The Standard Operating Procedure (SOP) has been written to:

- Ensure clear procedures relating to the safe preparation and transfer of critical care infusions between theatres and critical care
- To maintain safety of the patient during transfer
- Provide consistency and continuity for nursing staff in the preparation of medication used to support the transfer of critical care patients between departments
- Provide consistency and continuity in the infusion devices used to safely deliver medication.
- To reduce the risk of incorrect preparation of critical care medication and need to switch infusion devices which can affect the continuation of essential treatment to the patient.

3. Scope

3.1. This Standard Operating Procedure (SOP) relates to the following staff groups who may be involved in the preparation and transfer of patients from theatres to critical care.

- Intensivists, Anaesthetic doctors
- Registered Nurses/Operating Department Practitioners working within theatres and theatre recovery

Note: there may be extreme circumstances where a request for infusions is made by the Consultant Intensivist for a patient outside of theatres and critical care. This would be to facilitate resuscitation or intra/inter hospital transfer. In these cases Appendix 1 can be used to ensure safety of medication preparation and transfer. The patient MUST be actively managed by either a Consultant Intensivist, anaesthetic doctor and/or an critical care nurse who are skilled and competent in medication administration, titration and management.

4. Location

4.1. This Standard Operating Procedure is only applicable to the following departments

- Theatre
- Critical Care
- Anaesthetics

5. Procedure

- Patient identified as needing Level 2 or 3 critical care bed. Confirmation of admission decided by Consultant Intensivist

- Critical Care nurse in charge notified of expected admission by Consultant Intensivist with an anticipated time for admission.

- Theatre Anaesthetist to request medication preparation prior to patient transfer to critical care using the 'transfer of critical care medication record chart' (appendix 1). Other specialist drugs may be required in addition to the standard prescription to support patient treatment. These will need to follow the standard preparation required by critical care and in line with the Trust Injectable Medicines Risk Assessment procedure (currently 'Medusa'), as per Trust Injectable Medicines Policy and associated Standard Operating Procedures. Prior to prescribing of additional medication confirmation must be sought from the Consultant Intensivist.

- Nursing staff on critical care are responsible for the preparation of medication. Checking of medication preparation should be completed by either a 2nd critical care nurse or ODP from theatre where the infusions are being commenced. This is to ensure Trust processes around 2nd checks are met and ensure safe custody of controlled drugs. This should be signed for on the transfer of critical care medication record chart following the Controlled Drugs Standard Operating Procedure and Preparing Injectable medicines standard operating procedure. For controlled drugs place of medication transfer needs to be stipulated in the controlled drug register held on critical care.

- The prepared medication should be set up using a Braun Perfusor and returned to Theatres with the completed transfer of critical care medication record Chart by the ODP or nurse who prepared/checked the medication. This **must** be signed by the registrant(s) who prepared the medication.

- It is the responsibility of the Anaesthetic doctor to connect, start and titrate the medication to the patient according to clinical need. The rate running on transfer to critical care **must** be stated on the transfer of critical care medication record Chart

- On arrival to critical care all medication signed on the 'transfer of critical care medication record chart' needs to be prescribed on the critical care chart and/or patient prescription chart.

- The transfer of critical care medication record chart **must** be filed in the patient's medical notes for audit and traceability of medication.

6. Associated Documentation

6.1. Northern Devon Healthcare NHS Trust Policies for :

- Injectable Medicines Policy and associated Standard Operating Procedures:
  - Preparing Injectable medicines standard operating procedure
  - Administering Injectable medicines standard operating procedure
- Medicines policy
  Controlled drugs policy and Standard Operating Procedure

APPENDIX 1

Patient Identification Label

Transfer of Critical Care Medication Record Chart

Critical Care, Theatres use only

Please refer to SOP for use outside of these departments

This document is to be used to support the safe preparation and transfer of critical care infusions between Theatres and Critical Care. On arrival to critical care **ALL** medication signed for below needs to be prescribed on the critical care charts and or the patient prescription chart. This document should be filed in the patient's medical notes for audit trail and traceability of medication.

Medication Request from (department):	**Transfer to (department):**

	Medication		Date	Time	Prescribed by: Sign/print name/GMC	Prepared by		Connected by	Date started	Time started	Rate ml/hr on transfer	Time infusion stopped	Wasted infusion
1	Drug	Propofol 2%											
	Preparation	50ml											
	Concentration	20mg/ml											
2	Drug	Remifentanil											
	Preparation	10mg in 50ml 0.9% saline											
	Concentration	200micrograms /ml											
3	Drug	Noradrenaline											
	Preparation	4mg made up to 50ml 5% Glucose											
	Concentration	80micrograms/ ml											

Please use overleaf for the prescribing of other specialist infusions. Please seek confirmation from the Consultant Intensivist prior to requesting other medications

Patient Identification Label

Medication Request from (department):	Transfer to (department):

	Medication		Date	Time	Prescribed by: Sign/print name/GMC	Prepared by		Connected by	Date started	Time started	rate ml/hr on transfer	Time infusion stopped	Wasted infusion
4	Drug												
	Preparation												
	Concentration												
5	Drug												
	Preparation												
	Concentration												
6	Drug												
	Preparation												
	Concentration												

Additional infusion for consideration

DRUG	Strength	Preparation	Notes
Adrenaline	1:1000	5mgs/5mls made up to 50mls with 5% Dextrose (100mcgs/ml)	CVC only
Amiodarone *loading dose*	300mgs	300mgs in 250mls 5% Dextrose to be given over 60 minutes	Peripheral or via CVC
Amiodarone *maintenance dose*	900mg	900mgs in 500mls 5% Dextrose over 24 hours	CVC only
Potassium Chloride	2mmols/ml	2mmols/ml neat 50mls	Run up to 20mmols/hr (10mls/hr) CVC only